Colchimil 0.6mg Tablet

Name and Strength of Active Substance
Each tablet contains Colchicine 0.6mg

Product Description
The product is a white, round, and biconvex uncoated tablets with no scores on either sides of the tablet.

Pharmacodynamics
Colchine acts against the inflammatory response to urate crystals by possibly inhibiting the migration of granulocytes into the inflamed area. This reduces the release of lactic acid and pro- inflammatory enzymes that occurs during phagocytosis and breaks the cycle that leads to the inflammatory response.

Pharmacokinetics
Colchicine can reach the peak plasma concentration within 2 hours from the dosing of oral administration. The drug will partially undergo deacetylation process in the liver. The remaining unchanged colchicine and its metabolites will be expelled via excretion in the bile and intestinal reabsorption. The compound of colchicine usually can be found in leucocytes, kidneys and the liver and spleen in high concentration. It will be expelled in faeces but 10 to 20% will be excreted in the urine and this proportion rises in patients with liver disorder.

Indication
For the treatment of acute gout and for the prophylaxis of recurrent gout and to prevent acute attacks during initial therapy with allopurinol and uricosuric drugs.

Recommended Dosage
Patients vary widely in their ability to tolerate the drug. Dosage should be varied according to body weight or individual reaction to colchicine toxicity.
<u>Acute gout</u>
1-2 tablets every 2 hours until the pain is relieved or until nausea, vomiting and diarrhea occurs. The usual amount of colchicine required in a course of therapy may range from 4-8 mg.
Pain and swelling usually abate within 12 hours and are usually gone within 48-72 hours.
<u>Prophylaxis</u>
Depending on the severity and patient’s response and tolerance, the dosage varies from 1 tablet daily 1-4 times weekly to 1 tablet daily.
<u>Pseudogout</u>
To relieve acute attack, 1-2 tablets every 2 hours until the pain is relieved or until nausea, vomiting and diarrhea occurs.
<u>Prophylaxis</u>
Depending on the severity and patient’s response and tolerance, up to 2 tablets daily.

Route of Administration
For oral administration. Tablets should be swallowed whole with a glass of water.

Contradictions
Colchicine is not suitable for:
- Patient who are hypersensitivity to the active substance or to any of the excipients might occur in patients which are:
  - Patients with blood dyscrasias
  - Pregnancy
  - Breastfeeding
  - Women of childbearing potential unless using effective contraceptive measures
  - Patients with severe renal or hepatic impairment

- Colchicine should not be used in patients undergoing hemodialysis since colchicine cannot be removed by dialysis or exchange transfusion.
- Colchicine is contradicted in patients with renal or hepatic impairment who are taking a P-glycoprotein (P-gp) inhibitor or a strong CYP34A4 inhibitor (see *Drug Interaction*).

Warnings and Precautions
Colchicine is potentially toxic so it is important not to exceed the dose prescribed. Colchicine has a narrow therapeutic window. The administration should be discontinued if toxic symptoms such as nausea, vomiting, abdominal pain and diarrhea occur. Colchicine may cause severe bone marrow depression (agranulocytosis, aplastic anaemia, and thrombocytopenia). The change in blood count may be gradual or very sudden. Aplastic anaemia in particular has a high mortality rate. Periodic checks of the blood picture are essential.
If patients develop sign and symptoms that could indicate a blood cell dyscrasia, such as fever, stomatitis, sore throat, prolonged bleeding, bruising, or skin disorders, treatment with colchicine should be immediately discontinued and a full haematological investigation should be conducted straight away.

Caution is advised in case of:
1) liver or renal impairment
2) cardiovascular disease
3) gastrointestinal disorders
4) elderly and debilitated patients
5) patients with abnormalities in blood counts

Co-administration with P-gp inhibitors and/or moderate or strong CYP3A4 inhibitors will increase the exposure to colchicine, which may lead to colchicine-induced toxicity including fatalities. If treatment with a P-gp inhibitor or a moderate or strong CYP3A4 inhibitor is required in patients with normal renal and hepatic function, a reduction in colchicine dosage or interruption of colchicine treatment is recommended.

This medicine contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Interaction with other medication
Colchicine is a substrate for both CYP3A4 and transport protein P-glycoprotein. In the presence of CYP3A4 or P-glycoprotein inhibitors, the concentrations of colchicine in the blood increase.

Potential risk of severe drug interactions, including death, in certain patients treated with colchicine and concomitant P-glycoprotein or strong CYP3A4 inhibitors such as clarithromycin, cyclosporin, erythromycin, calcium channel antagonists (e.g. Verapamil and Diltiazem), telithromycin ketoconazole, itraconazole, voriconazole, HIV protease inhibitors, calcium channel blockers (verapamil and diltiazem), disulfiram and nefazodone.

P-glycoprotein (e.g. cyclosporin, verapamil or quinidine) or strong CYP3A4 inhibitors (e.g. ritonavir, atazanavir, indinavir, clarithromycin, telithromycin, itraconazole or ketoconazole) are not to be used in patients with renal or hepatic impairment who are taking colchicine.

A dose reduction or interruption of colchicine treatment should be considered in patients with normal renal and hepatic function if treatment with a P-glycoprotein or a strong CYP3A4 inhibitor is required.

A 4-fold reduction of colchicine dosage is recommended when co-administered with a P-glycoprotein inhibitor and/or a strong CYP3A4 inhibitor. A 2-fold reduction in colchicine dosage is recommended when co-administered with a moderate CYP3A4 inhibitor.

In addition, substances such as cimetidine and tolbutamide reduce metabolism of colchicine and thus plasma levels of colchicine increase.

Grapefruit juice may increase plasma levels of colchicine. Avoid consuming grapefruit and grapefruit juice while using colchicine.

Reversible malabsorption of cyanocobalamin (vitamin B12) may be induced by an altered function of the intestinal mucosa.

The risk of myopathy and rhabdomyolysis is increased by a combination of colchicine with statins, fibrates, ciclosporin or digoxin.

Pregnancy and Lactation
Colchicine is contradicted during pregnancy and lactation.

Undesirable Effects / Adverse Reactions
Blood and lymphatic system disorders
Not known: Bone marrow depression with agranulocytosis, aplastic anemia and thrombocytopenia
Nervous system disorders
Not known: Peripheral neuritis, neuropathy
Gastrointestinal system disorders
Common: Abdominal pain, nausea, vomiting, and diarrhea
Not known: Gastrointestinal haemorrhage
Hepatobiliary disorders
Not known: Hepatic damage
Skin and subcutaneous tissue disorders
Not known: Alopecia, rash
Musculoskeletal and connective tissue disorders
Not known: Myopathy and rhabdomyolysis
Renal and urinary disorders
Not known: Renal damage
Reproductive system and breast disorders
Not known: Amenorrhoea, dysmenorrhea, oligospermia, azoospermia
Others
Angioneurotic oedema, epistaxis, chromosomal dysfunction

Overdose and Treatment
Colchicine has a narrow therapeutic window and is extremely toxic in overdose. Patients at particular risk of toxicity are those with renal or hepatic impairment, gastro-intestinal or cardiac disease and patients at extremes of age.

Symptoms may be delayed (3 hours on average): nausea, vomiting, abdominal pain, hemorrhagic gastroenteritis, volume depletion, electrolyte abnormalities, leukocytosis, and hypotension in severe cases. The second phase with life threatening complications develops 24 to 72 hours after drug administration: multisystem organ dysfunction, acute renal function, and confusion, coma, ascending peripheral motor and sensory neuropathy, myocardial depression, pancytopenia, dysrhythmias, respiratory failure, and consumption coagulopathy. Death is usually a result of respiratory depression and cardiovascular collapse. If the patient survives, recovery may be accompanied by rebound leukocytosis and reversible alopecia starting about one week after the initial ingestion

Treatment:
No antidote available

Elimination of toxins by gastric lavage within one hour of acute poisoning

Consider oral activated charcoal in adults who have ingested more than 0.1mg/kg bodyweight within 1 hour of presentation and in children who have ingested any amount within 1 hour of presentation

Hemodialysis has no efficacy

Close clinical and biological monitoring in hospital environment

Symptomatic and supportive treatment: control of respiration, maintenance of blood pressure and circulation, correction of fluid and electrolyte imbalance

The lethal dose varies widely (7 – 65mg single dose) for adults but is generally about 20mg.

Effect on Ability to Drive and Use Machine
No details are available regarding the influence of colchicine on the ability to drive and use machines. However, the possibility of drowsiness and dizziness should be taken into account.

Storage Condition
Store below 30^{o}C and protect from light.

Pack Size
1 x 10's tablets in a box
10 x 10's tablets in a box
50 x 10's tablets in a box
100 x 10's tablets in a box

Name and Address of Manufacturer
The United Drug (1996) Co., Ltd.
208 Romklao Road,
Minburi, Bangkok 10510,
Thailand

Name and Address of Marketing Authorization Holder
Milrin Pharmaceutical Co. (M) Sdn. Bhd.
125-L, 5th Floor, Wisma Getah,
Macalister Road, 10400
Penang, Malaysia.

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